

BVD

Belantamab mafodotin, bortezomib and dexamethasone is a cancer drug treatment. It is used to treat myeloma.

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What is BVD?

BVD is a combination of cancer drugs. It is used to treat a type of blood cancer called [myeloma](#). BVD stands for the initials of the drugs used:

- Belantamab mafodotin (B) is an [antibody-drug conjugate](#)[®]. This is a type of targeted therapy with chemotherapy. It is also called Blenrep[®].
- Bortezomib is a type of targeted therapy called a cancer growth inhibitor. It is also called Velcade[®] (V).
- Dexamethasone (D) is a [steroid](#).

It is best to read this information with our general information about [cancer drug treatments](#) and the [type of cancer](#) you have.

Your cancer team will talk to you about this treatment and its possible side effects before you agree ([consent](#)) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the [electronic Medicines Compendium \(eMC\)](#) website, which has patient information leaflets (PIL) for individual drugs.

How treatment is given

You have belantamab mafodotin as a drip (infusion) into a vein (intravenously). You have it in a chemotherapy day unit or clinic as an outpatient.

You have bortezomib as an injection under the skin (subcutaneously). You have the injections in the day unit or clinic to start with. Some people may be trained to do the injections themselves at home.

Dexamethasone is a tablet that you take at home.

During your course of treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- chemotherapy nurse or specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

During treatment, you will have tests to check that it is safe for you to have treatment. This includes:

- a blood test before each belantamab mafodotin infusion
- eye tests before at least the first 4 treatments.

Most people have some eye or eyesight changes during treatment. These are usually temporary and mild. To help prevent and reduce eye side effects:

- your cancer team will give you artificial tear eye drops to use at least 4 times a day during treatment – follow advice from your doctor, nurse or pharmacist about using these
- do not wear contact lenses during the course of treatment.

You will meet with a doctor, nurse or pharmacist before you have treatment. They will talk to you about your test results and ask how you have been feeling. If your test results are okay, the pharmacy team will prepare your cancer drugs.

Your course of treatment

Your cancer team will discuss your treatment plan with you. They may give you a copy of the plan to take home.

You usually have a course of several cycles of treatment over a few months. Each cycle of BVD is 21 days long.

You have belantamab mafodotin on day 1 of each cycle.

You usually have bortezomib on day 1, 4, 8 and 11. But this can vary.

You take dexamethasone tablets:

- in the morning before each bortezomib injection
- in the morning the next day.

After 8 cycles, you only have belantamab mafodotin. You usually continue treatment for as long as it is working for you and any side effects can be managed.

Your cancer team may also give you anti-sickness drugs and other medicines to take home. Take all your capsules or tablets exactly as they tell you to.

Having belantamab mafodotin as a drip

Your nurse will usually give you anti-sickness drugs before the cancer drugs. You have belantamab mafodotin as a drip over 30 minutes. You have it through one of the following:

- a [cannula](#) – a short, thin tube the nurse puts into a vein in the arm or hand
- a [central line](#) – a fine tube that goes under the skin of the chest and into a vein close by
- a [PICC line](#) – a fine tube that is put into a vein in the arm and goes up into a vein in the chest
- an [implantable port](#) (portacath) – a disc that is put under the skin on the chest or arm and goes into a vein in the chest.

Having bortezomib injections

You have bortezomib as an injection into your thigh or tummy (abdomen). You have the injection in a different place each time. Tell your doctor, nurse or pharmacist if the skin in the injection site becomes:

- painful
- swollen
- red, if you have white skin
- darker, if you have black or brown skin.

Taking dexamethasone tablets

Steroids can irritate the stomach lining. If you have indigestion or pain in your tummy, tell your doctor, nurse or pharmacist. They can prescribe drugs to help reduce stomach irritation.

Take your dexamethasone with food to help protect your stomach. Some steroid tablets are coated to help reduce irritation.

Some people find it more difficult to sleep while taking steroids. Taking your dexamethasone in the morning may help.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Always tell your doctor, nurse or pharmacist about any side effects you have. They can give you:

- drugs to help control some side effects
- advice about managing side effects.

It is important to take any drugs exactly as explained. This means they will be more likely to work for you.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Side effects while treatment is being given

Some people may have side effects while they are having a cancer drug or shortly after they have it:

Allergic reaction

Some people have an allergic reaction while having this treatment. Signs of a reaction can include:

- feeling hot or flushed
- shivering
- itching
- a skin rash
- feeling dizzy or sick
- a headache
- feeling breathless or wheezy
- swelling of your face or mouth
- pain in your back, tummy or chest.

Your nurse will check you for signs of a reaction during your treatment. If you feel unwell or have any of these signs, tell them straight away. If you do have a reaction, they can treat it quickly.

Sometimes a reaction happens a few hours after treatment. If you develop any of these signs or feel unwell after you get home, contact the hospital straight away on the 24-hour number.

Always call 999 if swelling happens suddenly or you are struggling to breathe.

Very common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

Effects on the eyes

This treatment often affects the eyes and eyesight. Side effects are usually mild and get better after treatment. Follow any advice from your doctor, nurse or pharmacist about caring for your eyes. Do not drive or use machinery if you have eyesight changes.

Contact the hospital on the 24-hour number if:

- you notice any change in your eyesight, such as blurred or double vision
- your eyes feel dry, itchy or irritated
- your eyes are painful or sensitive to light.

Your cancer team will give you advice and arrange any support you need. If needed, they will pause the belantamab mafodotin to help side effects improve.

Risk of infection

This treatment can reduce the number of white blood cells in your blood. These cells fight infection. If your white blood cell count is low, you may be more likely to get an [infection](#). A low white blood cell count is called neutropenia.

An infection can be very serious when the number of white blood cells is low. Your doctor may give you antibiotics and other drugs to take during this treatment. These may help reduce your risk of getting an infection.

But it is still important to get any infection treated as soon as possible. If you have any of the following symptoms, contact the hospital straight away on the 24-hour number:

- a temperature above 37.5°C
- a temperature below 36°C
- you feel unwell, even with a normal temperature
- you have symptoms of an infection.

Symptoms of an infection include:

- feeling shivery and shaking
- a sore throat
- a cough
- breathlessness
- diarrhoea
- needing to pass urine (pee) often, or discomfort when you pass urine.

It is important to follow any specific advice your cancer treatment team gives you.

Your white blood cell count will usually return to normal before your next treatment. You will have a blood test before having more treatment. If your white blood cell count is low, your doctor may delay your treatment for a short time, until your cell count increases.

You may be given a drug called [G-CSF](#). This encourages the body to make more white blood cells. You have it as a small injection under the skin.

Bruising and bleeding

This treatment can reduce the number of platelets in your blood. Platelets are cells that help the blood to clot.

If the number of platelets is low, you may bruise or bleed easily. You may have:

- nosebleeds
- bleeding gums
- heavy periods
- blood in your urine (pee) or stools (poo)
- tiny red, brown or purple spots that may look like a rash – these spots can be harder to see if you have black or brown skin.

If you have any unexplained bruising or bleeding, contact the hospital straight away on the 24-hour number. You may need a drip to give you extra platelets. This is called a platelet transfusion.

Anaemia (low number of red blood cells)

This treatment can reduce the number of red blood cells in your blood. Red blood cells carry oxygen around the body. If the number of red blood cells is low, this is called anaemia. You may feel:

- very low in energy
- breathless
- dizzy and light-headed.

If you have these symptoms, contact the hospital straight away on the 24-hour number. You may need treatment for anaemia. If you are very anaemic, you may need a drip to give you extra red blood cells. This is called a blood transfusion.

Diarrhoea

This treatment may cause [diarrhoea](#). Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. You may also have stomach cramps. If you have a stoma, it may be more active than usual.

If you are passing loose stools 3 or more times a day and this is not normal for you, contact the hospital as soon as possible on the 24-hour number. Follow the advice they give you about:

- taking anti-diarrhoea medicines
- drinking enough fluids to keep you hydrated and to replace lost salts and minerals
-

any changes to your diet that might help.

They might also ask you for a specimen of your stool to check for infection.

Feeling sick

You may feel sick during this treatment, but this is usually mild. Your doctor, nurse or pharmacist may prescribe you anti-sickness drugs to help prevent or control sickness. Take the drugs exactly as they tell you to. [If you feel sick](#), take small sips of fluid often and eat small amounts regularly. It is important to drink enough fluids.

If you continue to feel sick, or if you are sick (vomit) 1 to 2 times in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice.

Feeling tired

[Feeling tired](#) is a common side effect of this treatment. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can help you feel less tired.

If you feel sleepy, do not drive or use machinery.

Effects on the liver

This treatment may affect how your liver works. This is usually mild and goes back to normal after treatment ends. You will have blood tests to check how well your liver is working.

Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Raised blood sugar levels

This treatment may raise your blood sugar levels and increase the risk of diabetes.

You will have regular blood tests to check this.

Contact the hospital on the 24-hour number if you have these symptoms during treatment or after it ends:

- feeling thirsty all the time
- needing to pass urine (pee) more often than usual
- feeling more tired than usual.
- losing weight without trying.

If you have diabetes, your blood sugar levels may be higher than usual. You may need advice and support from your GP or diabetes team about managing this.

Mood changes

This treatment can affect your mood. Tell your doctor, nurse or pharmacist if you have:

- feelings of anxiety or restlessness
- mood swings (moods that go up and down)
- low mood or depression.

Effects on the lungs

This treatment can cause inflammation of the lungs. This is called pneumonitis. Contact the hospital straight away on the 24-hour number if you notice any of these changes during treatment or after it ends:

- breathlessness
- a cough that does not go away
- wheezing
- a fever, with a temperature over 37.5°C.

You should also tell them if any existing breathing problems get worse. You may have:

- tests to check your lungs
- treatment, if needed.

Other important information

Blood clot risk

Cancer and some cancer treatments can increase the risk of a [blood clot](#). Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Driving

Follow your cancer team's advice about driving during and after treatment. Side effects such as tiredness may affect your safety. Do not drive if you have eyesight changes or dizziness.

Hepatitis B reactivation

Tell your doctor, nurse or pharmacist if you have ever had hepatitis B (a liver infection). This treatment can make it active again. They will talk to you about this and will test you for it before you start your treatment.

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- [herbal drugs and complementary or homeopathic therapies](#)
- recreational drugs – for example, cannabis.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and [coronavirus \(covid\)](#). They may also recommend other vaccines, such as Shingrix[®] for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Sex

It is possible that small amounts of chemotherapy may be passed on through vaginal fluids or semen. If you have sex during this treatment, your cancer team will usually advise using condoms or a dental dam to protect your partner.

Contraception

Your doctor, nurse or pharmacist will advise you not to [get pregnant](#) or [make someone pregnant](#) while having this treatment and for some time afterwards. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

Breastfeeding

You are advised not to [breastfeed](#) while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Fertility

Some cancer drugs can affect whether you can [get pregnant](#) or [make someone pregnant](#). If you are worried about this, it is important to talk with your doctor before you start treatment.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Visit the [electronic Medicines Compendium \(eMC\)](#) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

[Cancer Information Development team](#)

Author

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help